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Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°168/MVE-BDBV/29/10/2026

Reporting date: October 29, 2026 Publication date: October 30, 2026

HIGHLIGHTS (last 24 hours)

Seventy-seven (77) new confirmed cases and 34 deaths (20 community deaths and 14 in Ebola Treatment Centers) of Ebola Virus Disease (EVD) were reported in the provinces of North Kivu (45), Ituri (28), Haut-Uélé (3), and Tshopo (1). A total of 430 new samples were analyzed; 77 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 17.9%. No new health zones were affected in the last 24 hours.

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
10351	5026	48,6%	1101	2 788	84,5%

Affected provinces and affected health zones

8 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele South Ubangi, and Nord Ubangi

67 out of 178 health zones affected (37.6%)

Summary of key points from the last 24 hours

At the end of the day on October 29, 2026, seventy-seven (77) **new confirmed cases** of Ebola virus disease (EVD) were reported in four provinces : **North Kivu (45 cases), Ituri (28 cases), Haut-Uélé (3 cases), and Tshopo (1 case).** Among these cases,

Twenty community deaths were reported **(11 in North Kivu, 8 in Ituri, and 1 in Haut-Uélé).** In addition, **14** confirmed deaths were recorded in Ebola Treatment Centers (10 in Ituri and 4 in North Kivu), bringing the total number of confirmed deaths reported during the period to **34 .**

In addition, 22 patients **(10 in North Kivu, 9 in Ituri and 3 in Tshopo)** were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the beginning of the year, the DRC has recorded **10,351 confirmed cases,** including **5,026 deaths,** representing a **case fatality rate of 48.6** %. **Ituri** province remains the epicenter of the epidemic, accounting for **71.0% of all reported confirmed cases** and **36.4% of new confirmed cases** notified nationwide, while North Kivu accounts for **58.4% of new cases** in the last 24 hours.

No new health zones have been affected in the last 24 hours.

Thus, 67 out of 178 health zones (37.6%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 29, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	28	7346	3 436	46,8%	28/36 (77,8 %)
North Kivu	45	2436	1 356	55,7%	17/34 (50,0 %)
Haut-Uélé	3	453	200	44,2%	7/13 (53,8 %)
Boss	1	92	27	29,3%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Nord Ubangi	0	6	1	16,7%	2/11(18,2%)
Total	**77**	**10 351**	**5 026**	**48,6%**	**67/178 (37,6%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 29, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7346**	**3436**	**46,8%**	**28**	**8**	**10**	**18**
Give	11	1	9,1%				0
Understand	25	7	28,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	220	49	22,3%				0
have	2	2	100,0%				0
Bunia	1990	686	34,5%	9	4	2	6
Ladies	41	10	24,4%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%				0
Flounder	2	0	0,0%				0
Kilo	39	13	33,3%				0
Team	193	118	61,1%				0
Liter	326	164	50,3%	2	1	1	2
Logo	14	9	64,3%				0
Fight	36	9	25,0%				0
distributed	5	1	20,0%				0
Washing	94	44	46,8%	1	1	0	1
Darkness	475	249	52,4%	3	0	3	3
Mangala	396	172	43,4%	2	0	0	0
Mongbwalu	717	346	48,3%				0
Nanny	340	197	57,9%				0
Nizi	846	308	36,4%	2	0	1	1
Legumes	139	43	30,9%				0

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Jungle	22	6	27,3%				0
Rwampara	1197	430	35,9%	8	2	3	5
Tchomia	86	27	31,4%	1	0	0	0
A valves	THAT	493	THAT				0
North Kivu	**2436**	**1356**	**55,7%**	**45**	**11**	**4**	**15**
Me	677	448	66,2%	16	5	0	5
Farm	25	9	36,0%	1	1	0	1
Butembo	341	199	58,4%	2	0	1	1
Ten	1	0	0,0%				0
Kalunguta	96	43	44,8%	1	1	0	1
Cut	673	412	61,2%	2	1	1	2
Source	3	2	66,7%				0
Kyondo	46	22	47,8%				0
Lubero	41	15	36,6%				0
Be careful.	115	39	33,9%	9	1	0	1
Alimbong	5	3	60,0%				0
Manguredjipa	16	5	31,3%				0
Masereka	41	13	31,7%	1	0	0	0
The museums	160	77	48,1%	2	1	0	1
Mutwanga	55	13	23,6%	6	0	0	0
Oicha	85	38	44,7%	5	1	2	3
Vuh	56	18	32,1%				0
Upper Uele	**453**	**200**	**44,2%**	**3**	**1**	**0**	**1**
Government Mangbetu	46	19	41,3%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	113	53	46,9%				0
Lining	111	63	56,8%	1	0	0	0
Beans	6	3	50,0%				0
Ordinary	173	60	34,7%	2	1	0	1
Boss	**92**	**27**	**29,3%**	1	0	0	0
Bafwasende	18	3	16,7%				0
Kabondo	20	2	10,0%				0
Funeral a	1	0	0,0%				0
Maquis - Kisangani	37	14	37,8%	1	0	0	0
Mangobo	10	7	70,0%				0
Boss	5	0	0,0%				0
Wanie - Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Nord Ubangi	**6**	**1**	**16,7%**	0	0	0	0
You are logged in.	3	1	33,3%				0
He returned.	3	0	0,0%				0
Total	**10351**	**5026**	**48,6%**	**77**	**20**	**14**	**34**

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*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.
NA : Not Applicable.

Table 3. Status of alerts notified by province for October 29, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Name of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	120	6	126	8	6	107	0	14	1
Bas Uélé	144	1	145	16	1	128	0	17	0
Ituri	1 141	65	1 206	138	65	989	2	202	131
North Kivu	1 182	34	1 216	143	33	1 039	1	176	134
Sud Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Nord Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Boss	97	0	97	3	0	94	0	0	0
Grand Total	**2 684**	**106**	**2 790**	**308**	**105**	**2 357**	**3**	**409**	**266**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

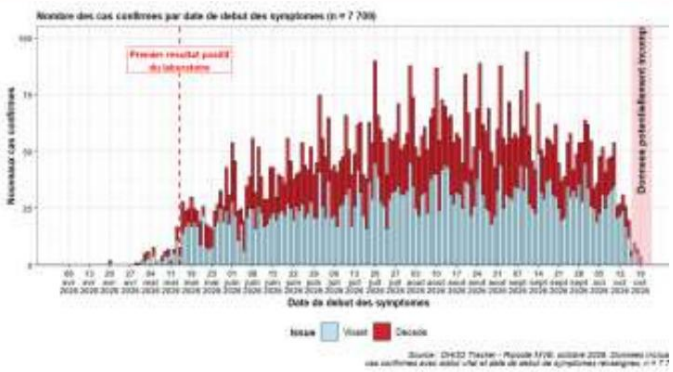
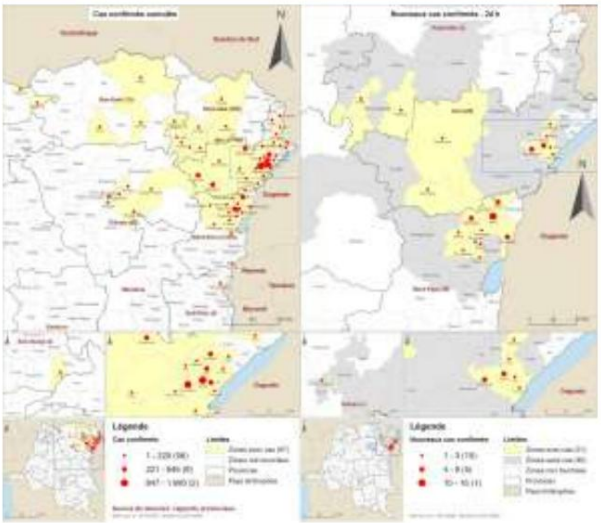


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1.COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars; launch in Kisangani, on October 29, 2026, of the Intra-Action Review (IAR) of the response against the 17th Ebola outbreak, under the chairmanship of the Chief of Staff of the Minister of Public Health, Hygiene and Social Welfare.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.2. Main Actions

As of October 29, 2026, **2,790 alerts** had been notified in the five provinces that submitted their data.
(Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu and Tshopo), including **2,684** alerts of people alive and **106 deaths ;** 2

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As of October 29, 2026, 2,790 alerts had been reported in the five provinces that submitted their data (Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu, and Tshopo), including 2,684 alerts of living persons and 106 of deaths; 2,773 were verified (99.4%). Of these, 413 (14.8%) were validated as suspected cases and 2,360 (84.6%) were invalidated. Of the 413 validated suspected cases, 409 (99.0%) were investigated and 266 were transferred to COVID-19 treatment centers (CT/CTE); 4 suspected cases remain uninvestigated (3 in Tshopo and 1 in Ituri) (Table 3). In Haut-Uélé, active case finding covered 96 of the 108 planned sites (88.9%) in 8 health zones.

Of these, 37,367 contacts were under follow-up in the six provinces that submitted their data; 31,573 were seen in the last 24 hours, representing a follow-up rate of 84.5%, below the required threshold of 95.0% and down from the previous day (87.2%). By province, the follow-up rate is 98.7% in Bas-Uélé (774/784), 97.3% in Sud-Ubangi (109/112), 89.3% in Ituri (13,366/14,973), 81.3% in Haut-Uélé (1,617/1,989, of which only 63.3% is in Wamba), 81.1% in Nord-Kivu (15,031/18,524) and 68.6% in Tshopo (676/985), where the health staff strike and attacks against teams are disrupting follow-ups.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

For October 29, 2026, four (4) alerts were notified to the PoE/PoC: 2 in Ituri, 1 in North Kivu, and 1 in Bas-Uélé. In Ituri, the live alert from the Kasenyi PoC (Tchomia Health Zone) was validated, investigated, and transferred to the Kasenyi CTE (100%; 1/1), and the body of a 42-year-old woman intercepted at the DGDA checkpoint (Mahagi Health Zone) was notified, validated, and swabed (100%; 1/1), with the outcome pending. In North Kivu, the live alert from the Kahongo PoC (Kyondo Health Zone), and in Bas-Uélé, the live alert from the PK2 PoC (Poko Health Zone), were invalidated after investigation. In South Ubangi, three live alerts reported at the REGIDESO point of entry (PoE) in the Zongo Health Zone were validated and then transferred to the Zongo General Referral Hospital (HGR) for treatment. However, these alerts are not included in the monitoring table, as the checkpoint is not yet operational.

Table 4. Status of key PoE/PoC indicators by province, as of October 29, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Upper Uele	Lower Uele	Global
Report completeness	98,4% (61/62)	95% (20/21)	56% (5/9)	100% (18/18)	75% (9/12)	64% (14/22)	88,2% (127/144)
Strategic PoC/PoE	[1]	THAT	THAT	THAT	THAT	THAT	[1]
Number of past travelers	159 278 94 555		7 117	23 761	24 738	3 842	313 291
Proportion of travelers screened	97%	98%	98%	99%	99%	98%	97%
% of travelers who washed their hands	97%	98%	98%	99%	99%	99%	97%
99% of travelers made aware		98%	100%	100%	100%	88%	99%
Number of alerts notified	02	01	THAT	THAT	THAT	01	4
Number of validated live alerts	01	THAT	THAT	THAT	THAT	THAT	[1]
Number of lifeless bodies intercepted	01	THAT	THAT	THAT	THAT	THAT	[1]
% of suspected cases transferred to CT/CTE	100% (1/1)	0%	0%	0%	0%	0%	100% (1/1)
% of lifeless bodies swabed/secured	100% (1/1)	0%	0%	0%	0%	0%	100% (1/1)
% of suspected cases investigated within 24 h	100% (2/2)	0%	0%	0%	0%	0%	100% (2/2)

2.2.2. Challenges

In North Kivu, the Butuhe PoC remains closed since the security incident of September 26, 2026, and the threat persists against service providers on the Butembo axis; **in Haut-Uélé,** 3 out of 12 PoCs (Salisa, Bombombi, and Bangwene) did not report due to lack of connectivity, and an ambulance is missing on the Bolebole road (Wamba Health Zone); **in Bas-Uélé,** completeness remains low (64%; 14/22) due to connectivity and mobility problems

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and the strike of service providers; **in South Kivu,** the PoEs of Kavimvira, Kalundu Port SNCC and Baraka still do not have isolation devices and the completeness is only 56% (5/9).

2.3. Laboratory

2.3.1. Main actions

- **Ituri:** 28 new positive results (20 living and 8 dead) out of 218 new samples analyzed (positivity: 12.8%).
- **North Kivu:** 45 new positive results (34 live and 11 deaths) out of 172 new samples analyzed (positivity: 26.2%.
- **Tshopo:** 1 new positive result (1 live) out of 6 new samples analyzed (positivity: 16.7%); 4 confirmed patients were negative on control and a pre-break of reagent (MVE buffer) is reported.
- **Haut-Uélé:** 3 new positive results (2 living and 1 dead) out of 19 new samples analyzed at the Isiro laboratory (positivity: 15.8%); in Bas-Uélé, 15 samples were tested at the Buta laboratory, all came back negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri :** 31 rings opened out of 35 (88.6%), 28 households decontaminated out of 39 (71.8%) and 11 FOSA out of 13 (84.6%), mainly in Bunia, Nizi, Mandima, Bambu and Mambasa; 47 EDS carried out out of 68 alerts (69.1%), 40 bodies swabés.
- **North Kivu :** 16 rings opened out of 24 (66.7%), 6 households decontaminated out of 10 (60.0%) and 16 ESS out of 19 (84.2%); 170 ESS follow-ups, 336 providers briefed, 7 triage units installed and 4 score card assessments carried out (average score: 47.5%); the training of 3 ECUMR teams has started in the Kalunguta health zone; 47 EDS carried out out of 47 alerts and 47 bodies swabed.
- **Tshopo:** decontamination of a household around the 86th confirmed case (Tshopo Health Zone), provision of 8 replacement mattresses to households and ESS around the cases, follow-up of 8 ESS in 4 Health Zones and briefing of 101 providers on the different PCI themes; screening and hand hygiene of approximately 250 participants in the Intra-Action Review; 11 EDS alerts received, no EDS carried out.
- **Haut-Uélé:** 1 out of 1 ESS decontaminated (100%) in Boma Mangbetu; 3 ESS assessed in Wamba (HGR: 80.7%; Wamba Clinic: 79.3%; CH Matari: 78.1%), equipped with functional triage units, and 16 nurses briefed on the functionality of DLM and waste management; start of training of 30 providers from the Isiro Health Zone on the IPC protocol (support Samaritan's Purse); 7 EDS carried out on 7 alerts (3 in Wamba and 4 in Pawa) and 7 bodies swabés.
- **Bas-Uélé:** 3 ESS monitored and supported (CS du Congo in Buta, CS Azli and HGR de Viadana), 7 service providers and 2 hygienists briefed on waste management, bio-cleaning and hand hygiene; no ring opened or EDS carried out, activities being disrupted by the strike of service providers in Buta, Ganga and Viadana.

2.4.2. Challenges.

In North **Kivu,** none of the six decontaminated households received kits (0/6), eight treatment rings remain closed, primarily in Oicha and Mabalako, and 40 epidemiological screenings (ESS) have been postponed until the following day. **In Ituri,** infection prevention and control (IPC) teams remain insufficient in Mangala, Mandima, Damas, and Mambasa; IPC supplies are lacking in Mandima, Niania, and Mambasa; and 13 swab failures were recorded. **In Bas-Uélé,** the strike has paralyzed activities in Buta, Ganga, and Viadana; 16 out of 20 priority health centers (HSCs) lack functional triage facilities, and none have mattresses. **In Haut-Uélé,** only four out of seven health zones (HZs) have submitted their data; IPC kits are lacking for HSCs; and IPC supervisors are not activated in Rungu, Dungu, Boma Mangbetu, and Gombari.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri,** 117 new admissions were recorded compared to 67 discharges, including 9 recoveries and 2 discharges without medical advice. Thus, 422 patients (147 confirmed and 275 suspected) are hospitalized, representing an occupancy rate of 40%; 398 out of 409 patients (97.3%) received hot meals, and all children under LNPE/SLM were treated.

In North **Kivu,** 108 new admissions (6 confirmed and 102 suspected) were recorded, compared to 70 discharges, including 10 recoveries and 2 discharges without medical advice. A total of 546 patients are currently receiving care, including 97 children.

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children under 5 years old) are taken care of, including 188 outside CTE/CT, for a CTE/CT occupancy rate of 87.3% (358/410).

- **In Tshopo,** 5 new admissions (1 confirmed and 4 suspected) were recorded against 5 discharges, including 3 recoveries and 2 non-cases; 19 patients (12 confirmed and 7 suspected) are hospitalized at the CTE Météo (occupancy rate: 52.8%; 19/36) and all received hot meals, as did the 51 PPLs followed.

- **In Bas-Uélé,** 9 new suspects were isolated in the ESS against 75 discharges, including 45 non-cases (Dakwa, Ganga and Bondo) and 30 discharges without biological confirmation for non-compliant samples (Viadana), bringing the number of suspects isolated in the ESS to 38, due to the lack of a functional CTE; **the data on the management of Haut-Uélé** were not transmitted.

2.5.2. Challenges

In Ituri, the blood shortage persists, 11 patients in Mandima have not received nutrition, 40 patients are critically ill, and capacity remains low in Nyankunde, Lolwa, Mandima, and Biakato. **In North Kivu,** Ebola Treatment Centers (ETCs) remain near capacity (87.3%), 188 patients are being treated outside of ETCs, and 19 are critically ill. In Tshopo, two confirmed cases who escaped in Bafwasende remain missing. In Bas-Uélé, the standardized ETC in Buta is still under construction, and none of the 38 isolated suspected cases are receiving nutritional support.

2.6. Vaccination

2.6.1. Main actions

In Ituri, vaccination of people living with HIV (PLHIV) using Ervebo (MSF support) and research on its effectiveness against BDBV-related Ebola virus disease (BBDV) are continuing: 112 people were vaccinated during the day (50 women and 62 men), bringing the cumulative total to 2,081 (865 women and 1,216 men). The Partners, EBO-PEP, and Oracle trials, as well as qualitative and operational surveys (Africa CDC support), are also ongoing. **In Bas-Uélé,** no vaccination data was reported for the day.

2.6.2. Challenges

In Bas-Uélé, the UCCs are still not operational and vaccines are not available in sufficient quantities; in Tshopo, the activation of the PEV continuity plan is still awaited.

2.7. Continuity of Care

2.7.1. Main actions • In Tshopo,

assessments of the Bethany Health Center (Mangobo) and the Maison Médicale Polyclinic (Makiso-Kisangani) revealed a non-effective triage area in both health centers and a shortage of second-line anti-tuberculosis drugs; only 9 out of 36 staff (25.0%) are vaccinated against Ebola at the Polyclinic and 3 out of 8 (37.5%) at the Bethany Health Center; in North Kivu, the baseline assessment of the Maboya Hospital and the Matanda Secondary Hospital revealed the absence of an emergency service, and only 3 out of 27 providers (11.1%) trained in infection prevention and control (IPC).

2.7.2. Challenges

Low data reporting from the pillar (no data from Ituri, Haut-Uélé, Bas-Uélé and Sud-Ubangi); decline in the use of curative, IMCI, ANC, maternity and vaccination services in the Cokis health area; assessed recovery plans for health services not yet implemented and health services assessments not carried out in the Lubunga health zone (Tshopo); absence of emergency services and absenteeism contingency plan at the Maboya hospital and the Matanda hospital (North Kivu).

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri,** 29 out of 36 feedback requests (80.6%) and 12 out of 12 incidents (100%) were resolved within 48 hours; the CREC supported the surveillance pillar in pre-listing 365 contacts around 18 confirmed cases, decontaminating 66 out of 66 households (100%), and the IPC in carrying out 27 out of 30 EDS (90.0%), announcing 7 out of 7 positive results and transferring 34 out of 42 suspects (81.0%); 18,559 home visits reached 64,669 people and 333 educational talks were organized for 6,453 people; the training of 17 supervisors on the integrated village-centered Ebola response (VCER) continues in Nizi and local leaders in Komanda were involved in the dialogue against resistance.

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- **In North Kivu,** 19,044 home visits reached 61,842 people, 405 educational talks brought together 8,034 people and 54 mass sessions reached 5,004 people; 532 out of 650 alerts (81.8%) were notified, 34 out of 43 feedbacks (79.1%) and 7 out of 15 incidents (46.7%) were resolved within 48 hours and 274 out of 4,750 people (5.8%) were involved in community actions; 50 pastors and 30 school leaders in Butembo were briefed and the CREC moderated the discharge of 5 healed people.

In Bas **-Uélé,** 614 home visits reached 1,709 people, 3 mass awareness campaigns reached 1,197 people, and 2 educational talks were held for 40 people in the health zones of Ango, Monga, Bondo, Likati, and Ganga. Joint supervision with the vaccination and Point of Contact pillars was conducted in Ango, and the single community alert (Bondo) was resolved. **In Haut-Uélé,** 302 home visits reached 1,594 people, 1,252 students were sensitized during the decontamination of schools in Isiro, and the 6 community alerts, 3 feedback requests, and 1 incident were resolved.

- **In Tshopo,** 318 home visits reached 2,003 people, 8 mass awareness campaigns reached 6,519 people and 5 educational talks reached 251 people, and 5 radio programs were hosted; provincial CREC actors and ZS hotspots were trained in infodemic management, 4 out of 4 feedbacks were resolved, 41 community alerts (34 in Makiso-Kisangani and 7 in Tshopo) were notified and CREC supported the discharge ceremony of 3 healed people.

2.9. Mental Health and Psychosocial Support (MHPSS)
2.9.1. Main actions

- **In Ituri,** 18 confirmed cases out of 45 (40.0%), 60 confirmed out of 110 and 107 suspected out of 158 under follow-up as well as 47 new suspected out of 117 have been taken care of, 13 households of separated and orphaned children out of 15 (86.7%) have been accompanied, 73 children are being followed up, 5 recovered out of 9 reintegrated and 32 results announced, including 18 positive.

- **In North Kivu,** 25 of the last 30 confirmed cases (83.3%), 215 confirmed cases under follow-up, 279 new suspects and 245 suspects under follow-up have been taken care of, 31 out of 34 (91.2%) separated and orphaned households of children have been supported, 17 recovered people have been reintegrated and 103 results have been announced, including 30 positive results; 517 people (183 men, 262 women and 72 children) have benefited from psychoeducation sessions.

- **In Tshopo,** all confirmed cases (1 new and 11 under follow-up) and the 8 suspected cases under follow-up have been taken care of, 2 out of 2 resistances have been lifted, 4 results have been announced and 507 people, including 383 students from EP Abakola (Lubunga) and 20 pregnant women in Mangobo, have benefited from psychoeducation.

- **In Bas-Uélé,** 1 new suspect benefited from an SMSPS intervention, 2 discharged recoveries are being monitored and 12 people participated in psychoeducation sessions; SMSPS data from Haut-Uélé were not transmitted.

2.9.2. Challenges

In Ituri, only 15 out of 28 affected health zones (53.6%) are covered, and coverage remains low around the most recent cases in Mandima (2/16) and Bunia (7/11). **In North Kivu,** coverage remains low around the most recent cases in 9 health zones, 10 affected health zones lack childcare facilities, and the number of trained health and social care personnel remains insufficient. **In Tshopo,** community health workers in the Wanie-Rukula health zone are not trained, and the health sector lacks communication units and transportation. **In Bas-Uélé,** psychosocial support for confirmed cases is lagging.

2.10. Logistics
2.10.1. Main actions

- **In North Kivu,** IPC and EDS supplies (body bags, Tyvek, gloves, face shields) and tents were delivered at the Mutwanga ZS and the delivery of inputs to Walikale continues.

- **In Tshopo,** 80 bags of Ringer lactate and 70 bags of physiological saline were provided to the CTE Météo and PCI inputs to the decontamination teams of Makiso-Kisangani and to the PEP team, and, for health products and medicines (PSM), the "Medicines" working group met in response to the shortage of second-line antituberculosis drugs, but 115 motorcycles are still needed for the ZS.

- **In Bas-Uélé,** the construction of the CTE at the HGR in Buta (50% of the work) and the rehabilitation of the DPS offices are continuing, but 140 liters of diesel fuel, computers, printers and scanners are lacking.

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2.11. Security

2.11.1. Main actions

- **In Tshopo,** faced with the persistence of acts of insecurity against the actors of the response and the escapes from Bafwasende, the SGI recommended a meeting with the Governor of the province and the transmission to the security services of the identity of a confirmed case of Rekapi who refuses his transfer to the CTE; **in North Kivu,** the threat persists against the providers of the PoCs of the Butembo axis, where the Butuhe PoC remains closed since September 26, 2026, and, **in Haut-Uélé,** against the teams of the Bolebole axis (ZS of Pawa and Wamba).

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

In North Kivu, 8 IMC staff members were briefed and signed the code of conduct in Beni, and 1,124 people, including 840 students from the Vingazi Institute, as well as 53 stakeholders, were made aware of the issue; **in Tshopo,** 171 people, including 30 in Kabondo and 141 in Mangobo, were sensitized on the PEAHS, the referral circuit and zero tolerance; **in Haut-Uélé,** 135 people were sensitized on the PSEA, the reporting circuit and the rights of victims in Isiro, Wamba and Boma Mangbetu.

2.12.2. Challenges

- Coverage of affected areas remains low: in North Kivu, only 6 out of 34 health zones (17.6%) know the reporting mechanism; in Haut-Uélé, awareness tools in French and Lingala are insufficient; in Tshopo, collection tools are lacking and the community complaint management mechanism is not yet installed in all the health zones of Kisangani; only 3 out of 8 health districts (37.5%) reported their PSEA activities in the last 24 hours.

MESSAGES CLES

No new health zones have been affected in the last 24 hours.

hours; 67 out of 178 health zones (37.6%) remain affected in eight

provinces.

In the last 24 hours, the epidemic has surpassed 10,351 cases

and 5,026 confirmed deaths recorded in eight provinces since the start of

the epidemic.

Seventy-seven (77) new confirmed cases were reported in four

provinces, including 45 in North Kivu, 28 in Ituri, 3 in Haut-Uélé and 1 in Tshopo.

Transmission remains particularly marked in the Beni ZS (16

cas), from Mabalako (9 cas), from Bunia (9 cas) and from Rwampara (8 cas).

Of the 34 confirmed deaths reported, 20 are community deaths

(11 in North Kivu, 8 in Ituri and 1 in Haut-Uélé). The contact tracing rate

stands at 84.5% (31,573/37,367), below the 95% threshold, with the rates

the lowest in Tshopo (68.6%), where the health workers' strike

This disrupts monitoring, particularly in North Kivu (81.1%). The situation underscores the need

to strengthen early detection, contact tracing, and dignified burials

is secure

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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